PO No :PO10004580929-438

Customer Name	: Ms.PRABHAVATHI	Collected Via	: TATA 1MG BANGALORE
Age/Gender	: 52Y 9M 17D /Female	Referred By	: Self
Lab Visit ID	: MGB2079809	Collection Date	: 18/Oct/2026 07:45AM
Barcode ID/Order ID	: D55688187 / 19241966	Report Date	: 18/Oct/2026 02:14PM
Sample Type	: Serum	Report Status	: Final Report

BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval	Method
LIVER FUNCTION TEST				
Liver Function Test				
Bilirubin-Total	**0.19**	mg/dL	0.3-1.2	Diazonium Salt
Bilirubin-Direct	< 0.10	mg/dL	0-0.5	Diazo
Bilirubin-Indirect	See Comment	mg/dL	0.2-0.8	Calculated
Protein, Total	6.93	g/dL	6.4-8.3	Biuret
Albumin	4.05	g/dL	3.5-5.0	Bromocresol Green
Globulin	2.9	g/dL	2.3 - 4.1	Calculated
A/G Ratio	1.41	Ratio	0.8 - 1.9	Calculated
SGOT (Aspartate Aminotransferase)	18	U/L	11-34	NADH w/o P-5'-P
SGPT (Alanine Transaminase)	22	U/L	0-34	NADH w/o P-5'-P
SGOT/SGPT	0.82	Ratio		Calculated
Alkaline Phosphatase	112	U/L	46-122	Para-Nitrophenyl Phosphate
Gamma Glutamyltransferase (GGT)	19	U/L	0-38	L-gamma-glutamyl-3-Carboxy-4-Nitroanilide

Note: Direct bilirubin value obtained is below the reportable range hence exact value could not be reported. Kindly note that Indirect bilirubin could not be calculated.

Comment:

- Raised ALT and AST indicate hepatocellular damage (e.g. viral or drugs etc). ALT is more liver-specific while AST is also found in heart, skeletal muscle, and kidney. Mild elevation (less than twice normal) often resolves on its own. Fatty liver disease (especially with metabolic syndrome) is a common cause in asymptomatic cases. Certain drugs (paracetamol, statins), herbal supplements, energy drinks, and antibiotics may also affect liver function.
- SGOT/SGPT Ratio: Typically <1 in healthy individuals (vary between 0.7-1.4; higher in women than men). High SGPT (ratio <1) seen in acute or chronic hepatitis, autoimmune disorders, medications, toxins while ratio >1 indicates alcoholic hepatitis, cirrhosis, metastasis or non-hepatic issues (hemolytic diseases, CVS disorders).
- Elevated Alkaline Phosphatase and GGT: Suggest cholestatic diseases (e.g. bile duct obstruction, primary biliary cirrhosis etc.) and can also be due to bone disease, pregnancy, chronic renal failure, malignancy, and congestive heart failure.
- High Bilirubin: Indicates jaundice due to increased RBC breakdown, liver damage (e.g., infections, toxins), or cholestasis (e.g., gallstones, tumors).
- High Protein Levels: Seen in dehydration (e.g., severe vomiting, diarrhea) or increased production (e.g., inflammation, hematopoietic neoplasms). Low protein and albumin: Result from impaired synthesis (liver disease), decreased intake,

NABL certificate and scope

This test has been performed at
TATA 1MG BANGALORE
No 607, Ground, 1st & 2nd Floor, 80 Feet Road, 6th Block, Koramangala, Bengaluru, 560095

Dr. Pillala Krishnaveni
MBBS MD (Biochemistry)
Consultant Biochemist
KMC 88194

Scan for digital copy

State of the Art Lab Facilities
Delhi · Gurgaon · Jaipur · Lucknow · Dehradun · Prayagraj · Mumbai · Pune · Ahmedabad · Indore · Nagpur · Bangalore
Hyderabad · Chennai · Visakhapatnam · Kolkata · Bhubaneswar · Angul · Patna · Ranchi · Guwahati

PO No :PO10004580929-438

Customer Name	: Ms.PRABHAVATHI	Collected Via	: TATA 1MG BANGALORE
Age/Gender	: 52Y 9M 17D /Female	Referred By	: Self
Lab Visit ID	: MGB2079809	Collection Date	: 18/Oct/2026 07:45AM
Barcode ID/Order ID	: D55688187 / 19241966	Report Date	: 18/Oct/2026 02:14PM
Sample Type	: Serum	Report Status	: Final Report

BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval	Method

tissue damage, malabsorption, or increased renal excretion.

*Please note the change in BRI.
*Please note test values may vary depending on the assay method used.

TATA 1mg | Labs

NABL certificate and scope

This test has been performed at
TATA 1MG BANGALORE
No 607, Ground, 1st & 2nd Floor, 80 Feet Road, 6th Block, Koramangala, Bengaluru, 560095

Dr. Pillala Krishnaveni
MBBS MD (Biochemistry)
Consultant Biochemist
KMC 88194

Scan for digital copy

State of the Art Lab Facilities
Delhi · Gurgaon · Jaipur · Lucknow · Dehradun · Prayagraj · Mumbai · Pune · Ahmedabad · Indore · Nagpur · Bangalore
Hyderabad · Chennai · Visakhapatnam · Kolkata · Bhubaneswar · Angul · Patna · Ranchi · Guwahati

PO No :PO10004580929-438

Customer Name	: Ms.PRABHAVATHI	Collected Via	: TATA 1MG BANGALORE
Age/Gender	: 52Y 9M 17D /Female	Referred By	: Self
Lab Visit ID	: MGB2079809	Collection Date	: 18/Oct/2026 07:45AM
Barcode ID/Order ID	: D55688187 / 19241966	Report Date	: 18/Oct/2026 04:51PM
Sample Type	: Serum	Report Status	: Final Report

IMMUNOLOGY

VIRAL MARKER SCREENING (HIV, HBSAG, ANTI-HCV)

Test Name	Result	Unit	Bio. Ref. Interval	Method
Hepatitis B Surface Antigen (HBsAg)				
HBsAg				
Result	0.24	S/CO	<1.0 : Non Reactive >=1.0 Reactive	CMIA
Interpretation	Non Reactive		Non Reactive	

Comment:

Result in S/CO	Remarks
< 1	Non-Reactive
>=1	Reactive

- The HBsAg Screening assay is used for the Qualitative detection of hepatitis B surface antigen (HBsAg) in blood.
- This assay is intended for screening, to prevent HBV transmission in blood recipients and to reduce risk of perinatal transmission of HBV. It can also be used as an aid in diagnosis of HBV infection.
- Non-Reactive test result indicates absence of Hepatitis B Surface Antigen.
- Repeatedly Reactive specimens should be confirmed by a Neutralization test or Nucleic acid amplification (NAT e.g. HBV DNA PCR) confirmatory test, before final diagnosis and initiation of treatment.

Note:

- False Positive HBsAg can occur within 30 days of receiving hepatitis B vaccine, biotin supplements, mouse monoclonal antibodies, and in presence of heterophilic antibodies.
- False Negative results may be seen in presence of various HBV mutants, or if the sample is collected in early phase of infection; this requires further additional testing.
- Results should be interpreted in conjunction with patient history, HBV DNA PCR and other hepatitis markers for diagnosis of acute and chronic infection.

CMIA-Chemiluminescent Microparticle Immunoassay /CLIA-Chemiluminescent immunoassay

Hepatitis C Virus (HCV) Antibody				
Anti HCV				
Result	0.13	S/CO	<1.0: Non-Reactive >=1.0: Reactive	CMIA
Interpretation	Non Reactive		Non Reactive	

Comment:

NABL certificate and scope

This test has been performed at
TATA 1MG BANGALORE
No 607, Ground, 1st & 2nd Floor, 80 Feet Road, 6th Block, Koramangala, Bengaluru, 560095

Dr. Pritha Aggarwal
MBBS, MD (Pathology)
Consultant Pathologist
Reg. No: 0000011

Scan for digital copy

State of the Art Lab Facilities
Delhi · Gurgaon · Jaipur · Lucknow · Dehradun · Prayagraj · Mumbai · Pune · Ahmedabad · Indore · Nagpur · Bangalore
Hyderabad · Chennai · Visakhapatnam · Kolkata · Bhubaneswar · Angul · Patna · Ranchi · Guwahati

PO No :PO10004580929-438

Customer Name	: Ms.PRABHAVATHI	Collected Via	: TATA 1MG BANGALORE
Age/Gender	: 52Y 9M 17D /Female	Referred By	: Self
Lab Visit ID	: MGB2079809	Collection Date	: 18/Oct/2026 07:45AM
Barcode ID/Order ID	: D55688187 / 19241966	Report Date	: 18/Oct/2026 04:51PM
Sample Type	: Serum	Report Status	: Final Report

IMMUNOLOGY

VIRAL MARKER SCREENING (HIV, HBSAG, ANTI-HCV)

Test Name	Result	Unit	Bio. Ref. Interval	Method

Result in S/CO	Remarks
< 1	Non-Reactive
>=1	Reactive

- This is a screening test to detect anti-HCV antibody using CMIA.
- The CDC recommends the use of method specific optimal signal -to-cut- off ratio (S/CO) in interpretation & reporting positive results. S/CO Ratio 1.0-5.0: Requires further supplemental tests for confirmation S/CO Ratio ≥ 5.0: High probability (95% or more) of being true positive.

Interpretation

- A Reactive HCV antibody test may indicate current or resolved infection or a biological false positive. To differentiate past, resolved HCV infection from biological false positives, another HCV antibody assay can be considered.
- A reactive test cannot differentiate between the stages of Hepatitis C viral infection (Acute/ Chronic / Resolved infection) nor is used to monitor the efficacy of treatment.
- A Non-Reactive test result indicates Hepatitis C virus infection is unlikely, but cannot be completely ruled out. For individuals potentially exposed to HCV in the last 6 months, it is advised to test for HCV RNA or conduct follow-up HCV antibody testing.

Note:

- All Reactive results require supplemental testing for confirmation with more specific assays like HCV RNA PCR or Immunoblot. HCV RNA PCR is recommended in all Reactive results to differentiate between past and present infection. A Reactive anti-HCV antibody result followed by a positive supplemental test (HCV RNA PCR) suggests active Hepatitis C infection.
- False Positive results are seen in autoimmune disease, rheumatoid arthritis, hypergammaglobulinemia, passive antibody transfers, liver disease, sexually transmitted disease (STD) and anti-superoxide dismutase.
- False Negative results are seen in early acute infection, immunosuppression & immuno-incompetent states.
- Results should be interpreted in conjunction with patient history and other hepatitis markers like HCV-RNA for diagnosis of acute or chronic infection.

Uses:

- Routine screening of low and high risk populations including blood donors.
- Prenatal Screening of pregnant women.

CMIA-Chemiluminescent Microparticle Immunoassay /CLIA-Chemiluminescent immunoassay

NABL certificate and scope

This test has been performed at
TATA 1MG BANGALORE
No 607, Ground, 1st & 2nd Floor, 80 Feet Road, 6th Block, Koramangala, Bengaluru, 560095

Dr. Pritha Aggarwal
MBBS, MD (Pathology)
Consultant Pathologist
Reg. No: 0000011

Scan for digital copy

State of the Art Lab Facilities
Delhi · Gurgaon · Jaipur · Lucknow · Dehradun · Prayagraj · Mumbai · Pune · Ahmedabad · Indore · Nagpur · Bangalore
Hyderabad · Chennai · Visakhapatnam · Kolkata · Bhubaneswar · Angul · Patna · Ranchi · Guwahati

PO No :PO10004580929-438

Customer Name	: Ms.PRABHAVATHI	Collected Via	: TATA 1MG BANGALORE
Age/Gender	: 52Y 9M 17D /Female	Referred By	: Self
Lab Visit ID	: MGB2079809	Collection Date	: 18/Oct/2026 07:45AM
Barcode ID/Order ID	: D55688187 / 19241966	Report Date	: 18/Oct/2026 04:51PM
Sample Type	: Serum	Report Status	: Final Report

IMMUNOLOGY

VIRAL MARKER SCREENING (HIV, HBSAG, ANTI-HCV)

Test Name	Result	Unit	Bio. Ref. Interval	Method
HIV Combo (Antigen And Antibody) Test				
Result	0.06	S/CO	<1.0 : Non Reactive >=1.0 Reactive	CMIA
Interpretation	Non Reactive		Non Reactive	

Comment:

Result in S/CO	Remarks
< 1	Non-Reactive
>=1	Reactive

- HIV Ag-Ab Combo is a 4 th generation qualitative assay for simultaneous detection of HIV p24 antigen and antibodies associated with HIV-1 and/or HIV-2 virus in human serum.
- This test is used as an aid in diagnosis of HIV-1/HIV-2 infection and also as a screening test for detection of HIV positivity in donated blood or plasma.
- A Non-Reactive result indicates that antibodies to HIV-1 and/or HIV-2 are not detected in the sample. This may suggest that the patient has not been exposed to HIV-1 or HIV-2, or that the sample was taken during the "window period", i.e. before detectable levels of antibodies have developed. Therefore, a non-reactive result does not rule out the possibility of HIV-1 and/or HIV-2 infection.
- A Reactive result suggests the possibility of HIV 1 or HIV 2 infection. However, it does not differentiate between type of antibody and antigen detected. All reactive results are tested by three different principles /techniques before being reported as HIV positive. A positive result indicates presence of either p24 antigen or antibody against HIV-1/2 or both.
- In case of "Indeterminate" result, repeat testing is advised after 2-4 weeks is advisable.
- Confirmatory tests with high specificity, like Western Blots and line immunoassays, or HIV RNA PCR are used in cases of positive/indeterminate/discordant results.

Note:

- Results should be interpreted in conjunction with patient history
- Rarely false negativity/positivity may occur. False positives may be seen in autoimmune disease, alcoholic hepatitis, primary biliary cirrhosis, leprosy, multiple pregnancies, and due to presence of rheumatoid factor and heterophile antibodies. False negatives may be seen during the Window period and during the end stage of the disease.
- **Disclaimer**: This is a Screening Test.
- Test values may vary depending on the assay method used.
- Sensitivity of the test is 100%. Specificity of the test is 99.85% with a confidence interval at 95% of (99.61-99.96%). (Reference: Alinity HIV Ag-Ab kit insert)

To avail Post test counseling, kindly reach us at care@1mg.com .

NABL certificate and scope

This test has been performed at
TATA 1MG BANGALORE
No 607, Ground, 1st & 2nd Floor, 80 Feet Road, 6th Block, Koramangala, Bengaluru, 560095

Dr. Pritha Aggarwal
MBBS, MD (Pathology)
Consultant Pathologist
Reg. No: 0000011

Scan for digital copy

State of the Art Lab Facilities
Delhi · Gurgaon · Jaipur · Lucknow · Dehradun · Prayagraj · Mumbai · Pune · Ahmedabad · Indore · Nagpur · Bangalore
Hyderabad · Chennai · Visakhapatnam · Kolkata · Bhubaneswar · Angul · Patna · Ranchi · Guwahati

PO No :PO10004580929-438

Customer Name	: Ms.PRABHAVATHI	Collected Via	: TATA 1MG BANGALORE
Age/Gender	: 52Y 9M 17D /Female	Referred By	: Self
Lab Visit ID	: MGB2079809	Collection Date	: 18/Oct/2026 07:45AM
Barcode ID/Order ID	: D55688187 / 19241966	Report Date	: 18/Oct/2026 04:51PM
Sample Type	: Serum	Report Status	: Final Report

IMMUNOLOGY

VIRAL MARKER SCREENING (HIV, HBSAG, ANTI-HCV)

Test Name	Result	Unit	Bio. Ref. Interval	Method

CMIA-Chemiluminescent Microparticle Immunoassay /CLIA-Chemiluminescent immunoassay

TATA 1mg | Labs

NABL certificate and scope

This test has been performed at
TATA 1MG BANGALORE
No 607, Ground, 1st & 2nd Floor, 80 Feet Road, 6th Block, Koramangala, Bengaluru, 560095

Dr. Pritha Aggarwal
MBBS, MD (Pathology)
Consultant Pathologist
Reg. No: 0000011

Scan for digital copy

State of the Art Lab Facilities
Delhi · Gurgaon · Jaipur · Lucknow · Dehradun · Prayagraj · Mumbai · Pune · Ahmedabad · Indore · Nagpur · Bangalore
Hyderabad · Chennai · Visakhapatnam · Kolkata · Bhubaneswar · Angul · Patna · Ranchi · Guwahati

PO No :PO10004580929-438

Customer Name	: Ms.PRABHAVATHI	Collected Via	: TATA 1MG BANGALORE
Age/Gender	: 52Y 9M 17D /Female	Referred By	: Self
Lab Visit ID	: MGB2079809	Collection Date	: 18/Oct/2026 07:45AM
Barcode ID/Order ID	: D55688189 / 19241966	Report Date	: 20/Oct/2026 04:57PM
Sample Type	: Whole Blood	Report Status	: Final Report

SEROLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
Quantiferon-TB Gold plus (Interferon Gamma Release Assay)				
TB 1 antigen minus Nil	0.06	IU/ml	<0.35	EIA
TB 2 antigen minus Nil	0.04	IU/mL	< 0.35	
Mitogen minus Nil tube	>10	IU/ml		
Nil	0.03	IU/ml		
Final result	Negative			

"No significant interferon-gamma response detected. A negative result does not exclude tuberculosis, particularly in early infection or immunocompromised individuals."

Comment:

Interpretation

Nil (IU/ml)	TB1 minus Nil (IU/ml)	TB2 minus Nil (IU/ml)	Mitogen minus Nil (IU/ml)	QFT-Plus Result	Report/interpretation
≤8.0	≥0.35 and ≥25% of Nil	Any	Any	Positive	*M. tuberculosis* infection likely
	Any	≥0.35 and ≥25% of Nil			
	<0.35 or ≥0.35 and <25% of Nil	<0.35 or ≥0.35 and <25% of Nil	≥0.50	Negative	*M. tuberculosis* infection NOT likely
	<0.35 or ≥0.35 and <25% of Nil	<0.35 or ≥0.35 and <25% of Nil	<0.50	Indeterminate	Likelihood of *M. tuberculosis* infection cannot be determine
>8.0	Any				

This is an indirect assay for Mycobacterium tuberculosis infection & is intended for use in conjunction with clinical findings and other diagnostic tests.

NABL certificate and scope

This test has been performed at
TATA 1MG OKHLA
2nd Floor, B-225, Okhla Phase I, Okhla Industrial Estate, New Delhi, Delhi 110020

Ranjan yadav
Dr. Ranjan Yadav
MBBS, MD (Laboratory Medicine),
Consultant Physician, Laboratory Medicine,
Reg No: 91820,

Scan for digital copy

State of the Art Lab Facilities
Delhi · Gurgaon · Jaipur · Lucknow · Dehradun · Prayagraj · Mumbai · Pune · Ahmedabad · Indore · Nagpur · Bangalore
Hyderabad · Chennai · Visakhapatnam · Kolkata · Bhubaneswar · Angul · Patna · Ranchi · Guwahati

PO No :PO10004580929-438

Customer Name	: Ms.PRABHAVATHI	Collected Via	: TATA 1MG BANGALORE
Age/Gender	: 52Y 9M 17D /Female	Referred By	: Self
Lab Visit ID	: MGB2079809	Collection Date	: 18/Oct/2026 07:45AM
Barcode ID/Order ID	: D55688189 / 19241966	Report Date	: 20/Oct/2026 04:57PM
Sample Type	: Whole Blood	Report Status	: Final Report

SEROLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method

Note:

- For diagnosing or excluding tuberculosis disease and assessing the probability of Latent Tuberculosis infection (LTBI), requires a combination of epidemiological, historical, medical, and diagnostic findings that should be taken into account.
- The magnitude of the measured IFN-γ level cannot be correlated to the stage or degree of infection, level of immune responsiveness, or likelihood for progression to active disease.
- False-negative results can be due to stage of infection (e.g., sample taken prior to the development of cellular immune response), co-morbid conditions that affect immune functions, low lymphocyte counts, reduced or absent activation of immune response to TB antigens or other immunological variables. **CDC recommends repeat test after 8-10 weeks in case of high suspicion of tuberculosis.**
- False positive results can be seen in patients suffering from M. marinum, M. szulgai, M. kansasii.
- In children less than 5 years of age, Tuberculin skin test (TST) is recommended over IGRA. TB Gold Plus should be performed concurrently or within 3 days after placing a skin test to avoid the boosting phenomenon. The systematic boosting of the IGRA assay response is seen in TST- positive individuals when it is performed 2 to 4 weeks after TST. It is advisable that the sample for TB Gold Plus should be drawn on the same day of vaccination or 4 to 6 weeks after the administration of a live-virus vaccine.

Quantiferon-TB Gold Plus is a recommended screening test for those patients being placed on biologic treatment and other immunosuppression therapy. In patients suffering from LTBI, it is critical to screen for TB infection prior to initiation of immunosuppression treatment, including biologic agents for autoimmune diseases as introduction of biologics can cause a latent tuberculosis infection (LTBI) to activate due immunosuppression. QFT-Plus has two distinct TB antigen tubes: TB Antigen Tube 1 (TB1) and TB Antigen Tube 2 (TB2). Both tubes contain peptide antigens from the MTB-complex-associated antigens, ESAT-6 and CFP- 10. Both the TB1 tube and TB2 tubes contain peptides from ESAT-6 and CFP-10 that are designed to elicit CMI responses from CD4+ T-helper lymphocytes; the TB2 tube contains an additional set of peptides targeted to the induction of CMI responses from CD8+ cytotoxic T lymphocytes.

***** End Of Report *****

Disclaimer:
1. The reported results based on laboratory investigation, are only for the purposes of diagnosis and should be clinically correlated and interpreted by the referring physician/ medical practitioner. For any queries relating to the reported results, you may write to our customer support team on care@1mg.com
2. It is presumed that the tests performed are, on the specimen / sample being to the patient named or identified and the verifications of particulars have been confirmed by the patient or his / her representative at the point of generation of said specimen.
3. The reported results are restricted to the given specimen only. Results may vary from lab to lab and from time to time for the same parameter for the same patient (within subject biological variation).
4. The patient's details along with their results in certain cases like notifiable diseases and as per local regulatory requirements will be communicated to the assigned regulatory bodies.
5. The patient samples can be used as part of internal quality control, test verification, data analysis purposes within the testing scope of the laboratory.

NABL certificate and scope

This test has been performed at
TATA 1MG OKHLA
2nd Floor, B-225, Okhla Phase I, Okhla Industrial Estate, New Delhi, Delhi 110020

Ranjan yadav
Dr. Ranjan Yadav
MBBS, MD (Laboratory Medicine),
Consultant Physician, Laboratory Medicine,
Reg No: 91820,

Scan for digital copy

State of the Art Lab Facilities
Delhi · Gurgaon · Jaipur · Lucknow · Dehradun · Prayagraj · Mumbai · Pune · Ahmedabad · Indore · Nagpur · Bangalore
Hyderabad · Chennai · Visakhapatnam · Kolkata · Bhubaneswar · Angul · Patna · Ranchi · Guwahati

PO No :PO10004580929-438

Customer Name	: Ms.PRABHAVATHI	Collected Via	: TATA 1MG BANGALORE
Age/Gender	: 52Y 9M 17D /Female	Referred By	: Self
Lab Visit ID	: MGB2079809	Collection Date	: 18/Oct/2026 07:45AM
Barcode ID/Order ID	: D55688189 / 19241966	Report Date	: 20/Oct/2026 04:57PM
Sample Type	: Whole Blood	Report Status	: Final Report

SEROLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method

6. This report is not valid for medico legal purposes. It is performed to facilitate medical diagnosis only.
7. Pregnant women should seek guidance from a qualified obstetrician as test parameters may vary during pregnancy

TATA 1mg | Labs

NABL certificate and scope

This test has been performed at
TATA 1MG OKHLA
2nd Floor, B-225, Okhla Phase I, Okhla Industrial Estate, New Delhi, Delhi 110020

Ranjan yadav
Dr. Ranjan Yadav
MBBS, MD (Laboratory Medicine),
Consultant Physician, Laboratory Medicine,
Reg No: 91820,

Scan for digital copy

TATA 1mg | Labs

TATA

ENSURING ACCURACY IN EVERY SINGLE REPORT

Following a 3-step review process:



Advanced systems & cutting-edge technology analyze results with precision



Experienced lab experts and technicians conduct comprehensive reviews



Each report undergoes rigorous medical scrutiny & is signed off by a doctor

Have concerns regarding the report?

Reach out to our care team

care@1mg.com **1800-212-2323**



Accurate Testing, Assured Quality: Diagnostics Precision

Stringent quality control
Measures meeting international norms of safety guidelines

Cutting-edge technology
Robust healthcare systems equipped with calibrated & well-maintained machinery

Experienced lab staff
30+ medical professionals with a collective experience of 100+ years

Verified test procedures
Highly standardized test procedures following CLSI* guidelines

External assessments
Thorough third-party assessment by authorized experts

Trained phlebotomists
Ensuring smooth sample collection experience & pre-analytical precision

Watch how we take care of your sample



Protect Yourself with Essential Vaccinations

1800-102-1618

Online Consultation

At Home Service

Trained Professionals

Book now

*T&C Apply

*Clinical & Laboratory Standards Institute

T&C Apply